

THE WHITE ARMY

Mentor -
Dr CR Bhat

Presented by -
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BMCRI

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Patient particulars

Name : A

Age : 40 years

Gender : Male

Address : Peenya 2nd stage , Bangalore

Occupation : Auto driver

Socio economic status : Lower middle class (modified KuppuSwamy Scale)

Date of admission :

Date of examination : 1.09.2021

Chief complaints

Distension of abdomen since 1 month

Yellowish discolouration of eyes and skin since 1 month

Swelling of both lower limbs – 15 days

Vomiting of blood – 1 episode, 8 days back

History of presenting illness

Patient was apparently normal one month ago when he noticed insidious onset of painless abdominal distension Which was gradually progressive to the current state. There was associated loss of appetite and difficulty in breathing on lying down since one week. Breathless was not associated with wheeze. Not associated with PND, palpitations or chest pain. No history of constipation.

Patient developed yellowish discolouration of eyes which was first noticed by his wife one month back and which gradually progressed to involve the whole body and associated with high colour urine. There is no associated generalised itching or pale stools

Patient developed leg Swelling bilaterally which was insidious in onset involving only ankle. There was no associated decrease urine output, Hematuria , facial puffiness

Patient complaints of vomiting of blood 8 days back , brownish in colour about 1glass (100ml), non projectile, not associated with food particles. No history of blood in stools, black tarry stools

There was no history of fever during the course of the illness

No history of tatooing, IV drug abuse

No history of use of herbal medicine

No history of altered sleep wake pattern, day time drowsiness, altered consciousness

Past history

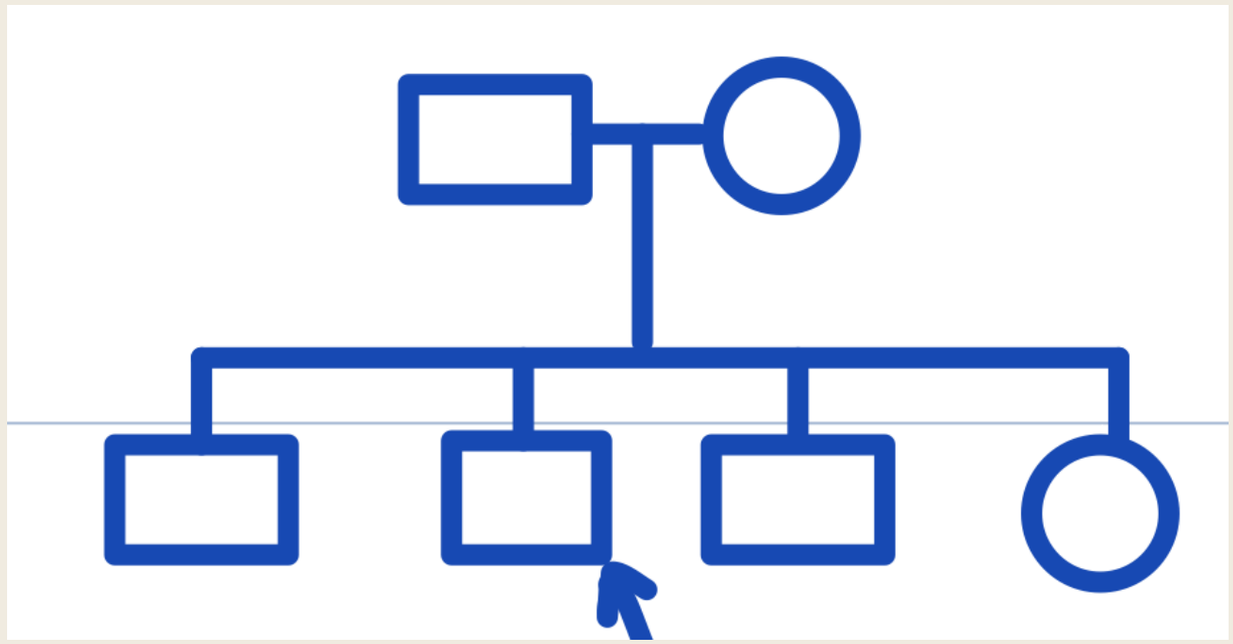
History of similar complaints 1 year back and was associated with yellowish discoloration of eye for which he was admitted to the hospital and treated.

Not a k/c/o Hypertension, diabetes mellitus, tuberculosis.

No H/O Blood transfusion, IV drug abuse, high risk sexual behaviour.

Family history

No similar complaints in the family



Personal history

Diet : vegetarian

Appetite : Reduced

Sleep : disused due to breathless since 1 week, needs 4-5 pillows

Bowel movements and bladder : Regular and normal

Habits : Alcoholic since 20 years (1 quarter whisky 180ml/day) 7.2 units/day, left alcohol 3 months back

Smoking-10 cigarettes per day since 20 years, 10 pack years No other recreational drug abuse

Drug history

No known drug allergies

Not on any chronic medications

Summary

48 year old male known alcoholic since 20 years presented with complaints of distension of abdomen and yellowish discolouration of eyes and skin since 1 month, associated with bilateral pedal oedema since 15 days , 1 episode of blood in vomitus, with similar complaints 9 months back which was treated with diuretics and relieved.

I would like to consider Decompensated Acute on chronic liver parenchymal disease secondary to alcohol consumption, associated with features of portal hypertension like Ascites and Upper GI bleed.

Differential diagnosis

1.Chronic Budd chiari syndrome

2.Malignant ascites

3.Tubercular ascites

General physical examination

Here is a middle aged male , moderately built and poorly nourished, conscious, co-operative and alert. Well oriented to time, place and person.

Vitals :

Pulse - 76 beats /min, normal in rhythm, volume, no radio – radial delay , radio – femoral delay, all peripheral pulses felt , no vessel wall thickening.

Respiratory rate - 17 cycles/min, thoracoabdominal type .

Blood pressure – 118/78 mm Hg measured in right upper arm, supine position.

Temperature – 36.2 degree celsius (measured in axilla)

Pallor – not seen

Icterus – seen

Clubbing – absent

Cyanosis – not seen

Lymphadenopathy – not present

Oedema - bilateral lower limb pedal oedema (grade 1), pitting present

1 spider Nevis on chest and 4 spider Nevi on the upper back seen

Dupuytren’s contracture present

No alopecia

No bitot spots or keratomalacia

No parotid swelling

No loss of facial hair

No fetor hepaticus

No angular Chelitis, glossitis, stomatitis, bald, magenta or pale tongue

No loss of axillary hair

No asterixis

No leuconychia

No gynecomastia

Abdominal examination
Inspection

Shape – distended uniformly

Flanks – full

All the quadrants - moves correspondingly with respiration.

Umbilicus – Shifted downwards, everted

Skin – stretched

Dilated veins are seen around the umbilicus

No visible pulsation

No visible gastric, intestinal peristalsis

No scars and sinuses

Hernial orifices – normal

External genitalia appears normal

Palpation

PALPATION ;Inspectory findings are confirmed

In standing position few dilated veins around the umbilicus with blood flow away from the umbilicus

Superficial palpation – no local rise of temperature/ tenderness

DEEP PALPATION

No guarding / rigidity

No rebound tenderness

Liver is palpable by dipping method, 6 cm below the right costal margin in mid clavicular line , firm in consistency , nodular surface, sharp border , non tender

Spleen is palpable about 5cm below the left costal margin, non tender, notch is felt over the medial aspects, soft in consistency.

Kidneys are not palpable

Abdominal girth 86 cm

Percussion

Liver dullness

Upper border – 5th right inter costal space on midclavicular line

Lower border – dullness extends 6cm below the right costal margin in mid clavicular line

Liver span – 16cm on midclavicular line

Shifting dullness present and Fluid thrill – absent

Spleen –CASTELL's method : dull note on percussion in the left 9th intercostal space in the anterior axillary line on full inspiration.

Auscultation

Normal bowel sounds are heard

No venous hum heard

No arterial bruit

No bruit heard over liver or spleen

Per rectal examination not done

Other system examination

Cardiovascular system

S1 and S2 heard normally

No murmurs heard

No additional sound heard

Respiratory system

Normal vesicular breath sounds are heard

No added sounds

CNS

Higher mental functions normal

No flapping tremors / altered sensorium

No focal deficits

Summary

40 year old male known alcoholic since 20 years presented with complaints of distension of abdomen and yellowish discolouration of eyes and skin since 1 month, associated with bilateral pedal oedema since 15 days , 1 episode of blood in vomitus, with similar complaints 1 year back which was treated with diuretics and relieved.

On examination

Icterus, spider Nevi, dupuytrines contracture seen

There is generalised uniform distension of abdomen

Dilated veins seen around the umbilicus with direction flow away from umbilicus

Palpable liver with a span of 16cm , palpable spleen

Shifting dullness present

Provisional diagnosis

Decompensated Acute on chronic liver parenchymal disease with features of portal hypertension, with no features of Hepatic encephalopathy probably due to alcohol consumption.

Investigations

Complete Blood Count, Coagulation Profile

Serology for HIV, HbsAg, HBC

Stool for Occult Blood

Urine Routine

Liver Function Tests

Renal Function Tests

Ascitic Fluid Analysis

USG Abdomen

Upper GI Endoscopy

Treatment

Salt and fluid restriction

Abstinence from alcohol

Bed rest

Diuretics :- spironolactone- 100mg/day, furosemide-40mg/day

Paracentesis

Liver transplantation

For portal hypertension

Beta – blockers-Metaprolol

Isosorbide nitrates

Octreotide

Endoscopic band ligation of varices

Transjugular Intraheparic Porto Caval Shunt

Thank you